

Introduction

Medication errors in the pharmacy can occur anywhere in the distribution system through Prescribing, Repackaging, Dispensing, and Monitoring. It may occur due to many reasons such as poor communication, lack of training or job stress. In addition, some factors can be medication related due to its labeling and packaging or patient related for example language barrier.

Purpose

- To outline the role of Pharmacists in identifying and handling medication errors and initiating appropriate corrective and preventive measures.
- To highlight the role for multi-disciplinary review of medication errors to allow appropriate implementation and follow-up of change to prevent future medication errors.

Scope

- This document is applicable to Pharmacy staff in Royal Hospital.

Definitions

- **Medication error:** Any preventable event that may cause or lead to inappropriate medication use or patient harm while the medication is in the control of the healthcare professional. Such events may be related to professional practice, healthcare products, procedures and systems including: prescribing, order communication, product labeling and packaging, nomenclature, compounding, dispensing, distribution, administration, education, monitoring and use.
- **Near miss:** Any process variation that did not affect an outcome but for which a recurrence carries a significant chance of a serious adverse outcome.
- **Significant Medication Error:** Any medication error that if not prevented may cause significant harm to the patient (i.e. permanent harm or death)

Policy

1. All pharmacist should identify and handle medication errors and initiate appropriate corrective and preventive measures involved during prescribing, dispensing, administration, and monitoring.
2. Focal point for medication error shall report the identified medication electronically via (IRLS) in alshifa system.
3. Medication error team review the incidence and if needed root cause analysis (RCA) should be conducted thus allow appropriate monitoring and follow-up of the changes to prevent future medication errors.
4. Reported errors should not be used for punitive purposes as those errors often result from fault within the followed systems rather than exclusively from the staff performance or environmental factor but should be used to activate corrections and changes.

Procedure

1. Once a medication error identified, it should be properly documented and reported immediately
2. For clinically significant errors, an immediate verbal notice should be given to the concerned physician/nurse/pharmacy professional, and any necessary corrective and supportive therapy should be provided to the patient.
3. All medication errors will be forwarded to head of the department (Medical, Nursing, Pharmacy) for appropriate management and corrective actions in a timely manner.
4. Reports of clinically significant errors RCA should be conducted and reported.
5. Reported errors should not be used for punitive purposes as those errors often result from fault within the followed systems rather than exclusively from the staff performance or environmental factor but should be used to activate corrections and changes.

References

Title of book/journal/article/website	Author	Year of publication	Page
Pharmaceutical Care Policies & Procedures in MOH Health Unit		January 2019	
American Society of Health System Pharmacists (ASHP) guidelines on the prevention of medication errors		1993	